

Suggested Protocol — Quick Reference

Version 1.5 — Shareable Protocol Model — Use Case: 7yr Male, Level 3, ~25kg | See Appendix for weight-based dosing

Protocol Model: The patient profile below (7yr male, Level 3, ~25kg) is a suggested use case. All doses should be scaled by weight using the appendix in the full protocol. Individualize under medical supervision.

1. Shopping List — Everything to Buy

Use Case: Quantities shown are for a ~25kg child. Adjust proportionally by weight.

How to use: Check off items as you buy. Items marked with **Ph1** needed Month 1, **Ph2** = Month 4+, **Ph3** = Month 12+, **Adv** = specialist only.

Diet Essentials

Item	Qty	Phase
GFCF staples: rice, quinoa, GF oats, buckwheat	Stock	Ph1
Bone broth (chicken or beef)	7 cups/week	Ph1
MCT oil (liquid)	1 bottle	Ph1
Epsom salt (magnesium sulfate)	2kg bag	Ph1
A2 milk / goat milk (for trial after 6 wks)	Optional	Ph2

Foundation Supplements (Month 1)

Supplement	Dose	Brand Tip
Vitamin D3 (liquid drops)	3,000 IU/day	Seeking Health, Thorne drops
Omega-3 DHA (liquid)	750 mg/day	Nordic Naturals, Carlson
Magnesium glycinate	200 mg at bedtime	Doctor's Best, Pure Encapsulations
Zinc picolinate	15 mg with dinner	Thorne, NOW Foods
Probiotic (multi-strain)	50 billion CFU	Klaire Labs, Seeking Health
Digestive enzymes	1 cap with meals	Houston Enzymes, NOW Super Enzymes
NAC	250 mg 2x/day	NOW Foods, Thorne
Niacinamide (no-flush)	50 mg with breakfast	NOW Foods, Solgar
Selenium	50 mcg/day	Thorne, NOW Foods

Phase 2 Supplements (Month 4+)

Supplement	Dose	Phase
Methyl B12 (injection or sublingual)	1,000 mcg/3 days	Ph2
L-methylfolate	400-800 mcg/day	Ph2
TMG (betaine)	500-1,000 mg/day	Ph2
Liposomal glutathione	500 mg/day	Ph2
CoQ10 (ubiquinol)	100-200 mg/day	Ph2
L-carnitine	500 mg 2x/day	Ph2
Creatine monohydrate	2-3 g/day	Ph2
NR (nicotinamide riboside)	125 mg/day	Ph2
Apigenin	50 mg 2x/day	Ph2
Alpha-lipoic acid	100 mg/day	Ph2
Sulforaphane	10-50 mg/day	Ph2
Taurine	250-500 mg 2x/day	Ph2

Gentle Detox (Month 4-6)

Item	Dose	Phase
Chlorella (broken-cell)	2 g 2x/day	Ph2
Cilantro tincture	10-15 drops 2x/day	Ph2
Silica-rich water (Fiji/Volvic)	1 L/day	Ph2
Malic acid powder	500 mg 2x/day	Ph2

Pharmacy / Prescription (Doctor Only)

Medication	Dose	Phase
LDN (low-dose naltrexone)	1.5-4.5 mg at bedtime	Ph2
Melatonin	1-10 mg at bedtime	Ph1
Nystatin suspension / Diflucan	Per doctor	Ph2
DMSA (chelation agent)	10 mg/kg 3x/day	Ph3
ALA (with DMSA pulse)	100 mg/day	Ph3
Activated charcoal capsules	250 mg between meals	Ph2
Luteolin 100 mg / Quercetin 500 mg	As needed	Ph2

17 2. Phase 1: Weeks 1-4 — Foundation

Goal: Stabilize, test, establish diet, start supplements, set up therapy team.

Week	Diet	Supplements	Testing / Therapy	Sleep / Other
Week 1	Start dairy-free. Bone broth daily. Epsom baths 5x.	Start D3, Omega-3, Mg glycinate, Zn picolinate	Order all lab tests. Start ATEC baseline.	Melatonin 1mg. Start bedtime routine.
Week 2	Continue dairy-free. Add MCT oil ½ tsp 1x/day.	Add probiotic, digestive enzymes, NAC 250mg 1x	Set up ABA (20+ hrs/wk). Start AAC/PECS daily.	Increase melatonin to 2mg if needed.
Week 3	Start gluten-free. Continue everything.	Full protocol: D3, Omega-3, Mg, Zn, probiotic, enzymes, NAC 2x, niacinamide, Se	MTHFR test. Speech therapy starts. Sleep diary begins.	Start eye contact + stimming tracking.
Week 4	GFCF fully implemented. MCT oil ½ tsp 2x.	Assess tolerance. Adjust Mg if loose stools. Adjust Zn if nausea.	First stool sample sent. Review initial sleep data.	Weighted blanket trial. Vagal therapy daily.

 **Daily Routine (Weeks 1-4):** Morning → D3 + Omega-3 + niacinamide + Se with breakfast. MCT oil in food. Lunch → enzymes. Dinner → Zn with meal. Bedtime → Mg glycinate + melatonin. Epsom bath 5x/week. AAC practice daily.

17 3. Phase 1: Weeks 5-12 — Deepen Foundation

Focus	Action Items
Weeks 5-6	Titrate MCT oil up (¼ tsp increase every 5 days). NAC to 250mg 2x/day. Review lab results as they arrive.
Weeks 7-8	Lithium orotate 5mg (optional, for brain health). Review first-month tracking data. Adjust diet if picky eating.
Weeks 9-12	Finalize Phase 1: stable GFCF, stable supplements, therapy team in place. Prepare for Phase 2. Repeat porphyrins if abnormal.

 **Watch for:** Herxheimer reaction (days 3-7 of diet change: increased stimming, irritability, loose stools). This is normal. Reduce dose, DON'T stop.



4. Phase 2: Months 4-6 — Active Treatment Begins

Requires: Stable GFCF diet, good sleep (6+ hrs), therapy team in place.

Month	Methylation	Glutathione	Detox	GI / Immune
Month 4	Start Methyl B12 injections (1,000 mcg every 3 days). L-methylfolate 400 mcg. TMG 500 mg.	Start liposomal glutathione 500 mg/day. NAC → 500 mg 2x/day.	Start chlorella 500 mg, titrate up. Cilantro 5 drops 2x.	Antifungals if Candida detected. Start LDN 1.5 mg if doctor prescribes.
Month 5	Add SAME 200 mg if tolerated. Increase TMG to 1,000 mg.	Add ALA 100 mg. Add NR 125 mg + apigenin 50 mg 2x.	Chlorella at 2g 2x. Cilantro at 10-15 drops 2x. Silica water 1 L/day.	Activated charcoal 250 mg if yeast die-off symptoms. Luteolin + Quercetin if inflammation high.
Month 6	Add CoQ10 100 mg. L-carnitine 500 mg 2x. Creatine 2 g.	Continue all. Add sulforaphane 10 mg, taurine 250 mg 2x.	Malic acid 500 mg 2x. Infrared sauna 10 min 3x/week.	First chelation consult with MAPS/DAN! doctor. Prepare for DMSA.



5. Phase 2: Months 7-12 — Chelation + Intensified Therapy

Week	Chelation (DMSA/ALA Pulse)	Therapy (Maximized)	Monitoring
Pulse Day 1	DMSA 200-300 mg every 8h. ALA 100 mg. Extra water.	ABA (normal). AAC practice.	Loose stools expected. Push fluids.
Pulse Day 2	DMSA + ALA continue.	ABA 30-40 hrs/wk (peak brightening!)	Behavior often improves dramatically.
Pulse Day 3	Last DMSA dose. ALA continue.	Speech + ABA double sessions.	Watch for redistribution regression day 4-5.
Off Days 4-14	No DMSA. Zinc 15 mg, Se 50 mcg daily.	Standard therapy schedule.	Epsom baths daily. Extra fluids.

Pulse schedule: 3 days ON, 11 days OFF. Repeat 8-12 rounds minimum. Each pulse = 2-week cycle. Lab work monthly (liver, kidney, Cu:Zn).

Emergency — STOP treatment if: Allergic reaction (hives, swelling), severe lethargy, <3 voids in 24h, persistent vomiting, seizure, blood in stool/urine.

6. Phase 3: Months 12-24+ — Maintenance

Treatment	Frequency	Notes
DMSA pulses	1 round every 3 months	Reduce from monthly to quarterly
Methyl B12	Weekly injection	Lifelong if MTHFR mutation +
GFCF diet	Continue	Trial A2 milk at 18 mo if desired
Supplements	Maintenance doses	D3, Omega-3, Mg, Zn, NAD+, MCT
Antifungals	As needed	Pulse if yeast symptoms return
Sulforaphane	Maintenance 10 mg	Can continue long-term

7. Advanced Options (Specialist Only)

Option	Indication	Consider If
IVIG	Autoantibodies + (MAR ASD)	No improvement after 12 mo standard therapy
HBOT	Neuroinflammation, brain injury	GFAP/NfL not declining
FMT	Refractory GI after 6 mo GFCF	Stool markers still abnormal
Nicotine patch	Aggression/irritability (3rd-line)	12+ mo standard therapy, MAPS doctor, no CV risk



8. Weekly Schedule Template

Time	Mon	Tue	Wed	Thu	Fri	Sat	Sun
8 AM	Supplements + breakfast (GFCF + MCT)					Sleep in / relaxed breakfast	
9-12	ABA therapy (Pulse weeks: max intensity on D2-D3)					OT / Sensory	Park / Nature
12-1	Lunch (GFCF + MCT oil + enzymes) + AAC practice						
1-3	OT	Speech	ABA	Speech	OT	Floortime	Family
3:30	Snack + AAC practice + play						
4-5:30	Vagal	Sensory	Music	Park	Free play	Errands	Free play
5:30-6	Dinner (GFCF + MCT oil)						
6-7	Epsom salt bath + wind down						
7-8	AAC practice, calm activities, visual schedule review						
8 PM	Melatonin + Mg glycinate + weighted blanket → bed						

9. Monthly Tracking Summary

Track	How	Target
ATEC score	Free online form — monthly	Decreasing trend
Functional communications	Count AAC/PECS/pointing acts weekly	50+ acts/day
Bristol stool + sleep diary	Type 1-7 + hours — daily	Type 3-4, 8 hrs
Diet compliance	% CFGF meals — daily	90%+
Lab repeats	Porphyryns q6mo, thiol q3mo, GFAP q6mo	Normalizing

Quick Tips: (1) Freeze Omega-3 capsules — reduces fishy burps. (2) Mix MCT oil into warm food, not cold. (3) Cut nicotine patches? No — use child-appropriate dosing or pharmacy-compounded. (4) Epsom baths before bed = better sleep. (5) DMSA pulse days 2-3 are GOLDEN — book extra therapy. (6) Herx reaction = reduce dose but DON'T stop.